Editorial decision

72/100 · Major revision before submission. I retain the v0.16 score. The revised manuscript answers several specific review questions, including what happens when a duplicate is excluded and how often the model is wrong at fixed coverage. The experiment most likely to change the recommendation is still pending: blinded readers must establish whether the target finding actually disappeared after erasure and remained visible after a control edit.

Scores under the existing rubric

Dimension	Weight	Score / 5	Points
Novelty and significance	15	3	9
Methodological soundness	15	4	12
Validity of evidence	15	4	12
Statistical analysis	10	4	8
Baselines and fairness	10	4	8
Clinical relevance and safety	10	2	4
Reproducibility and transparency	10	4	8
Honesty and limitations	5	4	4
Reporting quality and clarity	5	4	4
References, ethics and declarations	5	3	3

What v0.17 settles

The abstract now defines ESG as the difference in absent-or-refer rates and reports both the 0/48 pneumothorax detection result and the fresh-external utility deficit versus Gemini (−0.36, 95% CI −0.50 to −0.23). Section 4.1 accounts for 507 main-test question-cases across 491 image files. Section 5.3 identifies the C4 pilot’s reuse of test cases and supplies seed-paired continuation results.

Review scope. I compared the supplied v0.17 manuscript with v0.16 and its scored review. The exclusion, fixed-coverage and seed-paired figures below are manuscript-reported. The new per-view records and cited supplementary output were outside this update. I checked internal consistency and arithmetic, without independently reproducing those analyses.

Corrections supported by the numbers

1 Reconcile the duplicate-image statements

Section 4.1 identifies one image shared between validation and fresh external testing. Section 8.5 still says no image recurs. Report the single overlap in both places. Excluding it shifts net utility from −0.053 to −0.046, a change of **+0.007**, contradicting the claimed 0.003 upper bound. Report the actual shifts in ESG (+0.002), covered-and-correct rate (+0.003), and utility (+0.007). Give the post-exclusion positive SIIM denominator alongside the 95 remaining SIIM cases. Pixel matching leaves same-patient, different-visit overlap unresolved.

2 Use the abstract’s wording consistently for C1 and C2

The highlight “DPO ties on ESG” and the conclusion “reaches the same evidence sensitivity” overstate the comparison. Use the revised abstract’s “no fresh-external ESG advantage over removal-based DPO was established.” The C1 comparison changed both shape and tool at one seed. “Matching the control edit was not shown to improve the primary outcome” fits the evidence. “The control edit need not match” implies a stronger result.

3 Report the paired training-duration comparison throughout

All three point estimates favor continuation on erased-lesion claims (−1.2, −8.0 and −8.5 percentage points). Two of three intervals exclude zero. Section 5.3 currently says the rate fell in two seeds. Update the C4 lead, Table 8 caption and abstract to use the paired analysis. The older 11.5% pooled continuation versus 16.3% seed-0 recipe comparison mixes summaries. At seed 1, continuation also reduced unedited-lesion CCR by 4.9 points (95% CI −7.9 to −2.0). These observations support testing duration with a pre-specified sensitivity floor.

4 Keep the fixed-coverage comparison visible

At 70% coverage on main-test answerable views, MedScope’s error rate is 10.7%, versus 2.3–5.5% for the three flagships. At full coverage the rates are 15.1% and 11.4–13.7%. This gives the reader a concrete trade-off alongside the 9.6% main-test erased-lesion claim rate. State how confidence ties were handled at the cutoff, and report the uncertainty of the paired difference if using it to rank models.

Source locations Abstract and highlights, §§4.1, 5.3, 5.5, 8.5 and 9, Tables 5, 8 and 10. The C4 paired-view objective missed its primary endpoint against the compute-matched control (12.6% vs 11.5% erased claims, difference +1.1 points, 95% CI −0.9 to +3.0).

Evidence that would earn 5/5

Novelty · 3/5 today · six weighted points available

Show when the paired control changes a real model choice. Table 5 already supplies a candidate: Qwen2.5-VL-7B claims fewer erased lesions than MedScope (5.5% vs 9.6%), yet has substantially lower ESG (0.146 vs 0.758) and lower covered-and-correct rate (0.144 vs 0.849). The erased-claims difference has $p = 0.11$. Present this as an observed ranking reversal that motivates a prospective test.

Test the value added by ESG over a credible simpler rule. Freeze three selection rules: erased claims alone, erased claims subject to a floor on unedited sensitivity, and the paired-control rule subject to the same floor. Specify the candidate models, tie handling and smallest worthwhile gain before opening a new cohort. Have blinded readers establish the correct paired outcomes. A repeatable improvement in model selection over the sensitivity-adjusted comparator, across a second source or unseen editor, would justify full measurement-novelty credit.

Methodological soundness · 4/5 today · three points available

Finish and size the reader validation. Retain the registered pack of 128 images, record reader specialty, agreement on clinical presence, uncertainty, residual signs, and whether the target survives the control edit. The current 32 erased images give a two-sided exact 95% upper bound of 10.9% even at zero residuals. An extension needs at least 36 independent erasures with zero residuals to clear 10%. Plan for a larger sample if residuals are expected.

Rescore clinically verified pairs and lock the confirmation. The current four arms use different cases. In an extension, adjudicate erasure and control views from the same case while keeping individual readers blind to the pairing. Report the full sample, verified pairs, uncertain cases, and sensitivity to their treatment. Keep all unedited positives in the detection analysis, show patient-grouped intervals across views, and evaluate the frozen model-selection rule on previously unseen patients.

What would change the recommendation

If blinded adjudication confirms the paired labels and the main effect survives rescoring and overlap exclusion, I would reconsider major revision for a measurement paper. If validity differs by finding, restrict the primary claim to supported strata and retain diffuse or ambiguous findings as stress tests. Complete reader ethics and consent reporting, the AI-use declaration and model revision identifier before submission. Full credit on both scored dimensions would bring the total to **81/100**, holding other rows fixed.